

RESEARCH ARTICLE



Planning and implementation of a community-wide tuberculosis screening intervention: a case study from Naujaat, Nunavut, Canada

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ABSTRACT

In Nunavut, public health management of tuberculosis (TB) remains a challenge due to social and economic inequities, geographic isolation, TB-related stigma, and distrust in healthcare linked to a history of colonialism. A TB outbreak was declared in Naujaat, Nunavut following an increase in reported cases starting in January 2023. In response to this outbreak, the Naujaat community-wide TB screening (CWS) clinic was held from April 15th to May 29th, 2024, with support from the Nunavut Tunavik Incorporated (NTI) leadership and community engagement teams. This article outlines the steps taken in the coordination and implementation of a large public health intervention in this remote, northern community. Despite significant logistical challenges, the CWS clinic screened 89.2% of the 1000-person target consisting of all previously unscreened individuals residing in the community. As a result, 31 individuals with latent TB infection and <5 active TB cases were diagnosed and offered treatment, representing nearly 4% of those undergoing 2-step testing. Although the screening outcomes suggested that community transmission was low at the time of community-wide screening, it nonetheless exceeded the WHO threshold of 0.5% prevalence for community-wide screening. Lessons learned through the Naujaat CWS can help inform future public health actions towards TB elimination in Nunavut and other circumpolar jurisdictions.

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Introduction

Geographic, historical and sociosanitary context

Nunavut is part of the Inuit Nunangat—the traditional homelands of Inuit – which in Canada stretches from northern Labrador through northern Québec, Nunavut, and parts of the Northwest Territories and the Yukon. Management of tuberculosis (TB) in Nunavut, Canada's largest and northernmost territory, has been a public health challenge for decades due to a complex mix of historic, systemic and logistic factors. With a population of 36,858 people spread across more than 1.8 million square kilometers, 85% of whom are Inuit [1], all Nunavut communities ($n = 25$) lack roads connecting to each other and to southern Canada. Communities rely on year-round air travel for delivery of most goods and services, which can be impacted by inclement weather during most of the year, particularly during the long winters. Social and economic inequities persist including housing shortages, resulting in widespread overcrowding, as well as food and financial insecurity. The overall scarcity in human resources for healthcare delivery, high staff turnover and lack of TB-specific training, combined with the complexity of TB management and the need to use culturally safe practices are all longstanding challenges [2]. Stigma, fear, and distrust resulting from past policies and practices, particularly the forced removal of approximately 4,500 Inuit from their families and communities to attend southern sanatoria for TB treatment from the 1950s to the 1970s – 1,000 of whom never returned home – continue to shape perceptions of healthcare and participation in TB screening and care [3,4].

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